

[PROVIDER-NAME]

[ADDRESS]

Our Ref: (T) MS/mp/ [MED-ID]

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division B

Care Group - Emergency & Specialist Medicine Medical Secretariat

F Level Centre Block, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME] , [ADDRESS], Totton, Southampton, [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnoses:

Ulcerative colitis leading to colectomy and pouch formation with pouch anastomosis 2008

Tendency to develop salt and water depletion

DVTs on long-term Warfarin

Epilepsy

Pouch dysfunction

This woman came for review in the Intestinal Failure Clinic today saying that she continues to have problems with her pouchitis. In reality when she had this checked under EUA a few weeks back, found to have a striking cuffitis rather than pouch inflammation per se. In view of this she benefit from the use of either 5-ASA or steroid suppositories and I have written under separate cover to suggest that she starts by trying Pentasa 1 gm bd. She should continue to use her intermittent antibiotics as necessary but I am hoping that she will get some improvement in her pouch problems with the need for less antibiotic usage.

Otherwise she recently had some tests showing that she has a high urinary sodium and I have suggested that she can cut her St Mark's solution to 0.5 litres daily. I am quite surprised that she gets salt and water depletion and do want her to just repeat a 9 am cortisol to check that her adrenal function is adequate. She has certainly had low adrenal function in the distant past.

Yours sincerely

[PROVIDER-NAME]

Consultant Gastroenterologist